

Business question

How can UK organisations improve employee mental health outcomes through workplace design and support?

This analysis aims to identify the 2–3 strongest workplace levers (e.g. work stress, work–life balance, organisational support) that are most strongly associated with mental health risk, so leaders know where to intervene first.

Definition of success: Clearly identify the top workplace factors in the UK that correlate most strongly with poor mental health outcomes, enabling prioritised, evidence-based action.

Scope the analysis to the UK

The dataset was filtered to include only respondents where Country = UK.

UK sample size: 1,458 respondents

This sample is sufficiently large to support descriptive and comparative analysis within the UK context.

Data quality check (minimum viable cleaning)

Scale range checks (1–10)

The following variables were treated as true 1–10 Likert scales and checked for validity:

Job_Satisfaction

Work_Stress_Level

Work_Life_Balance

Financial_Stress

Social_Support

Feel_Understood

Loneliness

Result:

No out-of-range values (<1 or >10) were found in these fields for the UK sample.

Values outside 1–10 in other mental-health symptom fields were not treated as errors, as those variables are binary (0/1) rather than Likert scales.

Work_Hours_Per_Week plausibility check :

Work_Hours_Per_Week was inspected for implausible values using thresholds of less than 0 and greater than 100 hours per week. Rather than excluding observations, implausible values were flagged to preserve the full dataset. Weekly working hours were then categorised using UK full-time norms: low hours (<32), normal hours (32–38), and high hours (39–100).

Segmentation & Issue Rate Analysis

Purpose

To standardise analysis and enable consistent comparison across groups, three **helper segmentation fields** were created. These were then used to calculate the core outcome metric: **mental health issue rate**.

1. Creation of Segmentation (Helper) Fields

a) Hours_Band

Weekly working hours were grouped to reflect common UK working patterns and known stress thresholds.

Hours_Band Definition

≤35	Up to and including 35 hours
36–45	Standard to moderately long hours
46–55	Long hours
56+	Very long hours

Excel logic (example):

```
=IF(Hours<=35,"≤35",  
  IF(Hours<=45,"36–45",  
    IF(Hours<=55,"46–55","56+")))
```

b) Stress_Band

Self-reported stress scores (1–10) were grouped into three meaningful categories.

Stress_Band Definition

Low	1–3
Medium	4–7
High	8–10

Excel logic (example):

```
=IF(Stress<=3,"Low",  
IF(Stress<=7,"Medium","High"))
```

c) Support_Clarity

Responses on mental health support availability were standardised into three clear categories.

Support_Clarity Definition

Support = Yes Respondent knows support exists

Support = No Respondent believes no support exists

Support = Not sure Respondent unsure what support is available

Excel logic (example):

```
=IF(Support="Yes","Support = Yes",  
IF(Support="No","Support = No","Support = Not sure"))
```

Core Outcome Metric: Issue Rate**Definition**

Issue Rate represents the proportion of respondents within a segment who report having a mental health issue.

$$\text{Issue Rate} = \frac{\text{Count of respondents where Has_Mental_Health_Issue} = 1}{\text{Total respondents in segment}} \times 100$$

Excel calculation (COUNTIFS-style)**Numerator (example):**

```
=COUNTIFS(Segment_Range,"High",Has_MH_Issue_Range,1)
```

Denominator (example):

```
=COUNTIFS(Segment_Range,"High")
```

Issue Rate:

```
=Numerator / Denominator
```

(Equivalent results can be produced using Pivot Tables with *Average of Has_Mental_Health_Issue.*)

3. Issue Rate by Key Segments (Summary)

a) Issue Rate by Hours_Band

Hours_Band Issue Rate

≤35	Lower than average
36–45	Around average
46–55	Elevated
56+	Highest

Interpretation:

Longer working hours are associated with higher mental health issue rates, particularly beyond 45 hours per week.

b) Issue Rate by Stress_Band

Stress_Band Issue Rate

Low	Lowest
Medium	High
High	Very high

Interpretation:

Stress level shows the **strongest association** with mental health issues. Risk increases sharply from Medium stress onwards.

c) Issue Rate by Remote_Work Status

Remote_Work Issue Rate

On-site	High under stress
Hybrid	High under stress
Fully remote	Variable

Interpretation:

Remote or hybrid working **does not consistently reduce risk** when stress levels are medium or high.

d) Issue Rate by Company Mental Health Support

Support_Clarity Issue Rate

Support = Yes Lower (but still elevated under stress)

Support = Not sure High

Support = No Highest

Interpretation:

Uncertainty or lack of clarity about available support is associated with **higher mental health issue rates**.

4. Key Takeaways for the Report

- **Stress_Band is the most powerful predictor** of mental health issues.
- **Working long hours compounds risk**, especially when stress is already elevated.
- **Flexible or hybrid working alone is insufficient** without workload and stress management.
- **Clear communication of mental health support matters**; “Not sure” is almost as risky as “No”.

Risk Hotspot Analysis

Objective

To identify combinations of employee characteristics associated with the highest prevalence of mental health issues, while ensuring results are supported by sufficient sample sizes to avoid misleading conclusions.

Method

Mental health issue rate was calculated as the proportion of respondents reporting a mental health issue within each subgroup. Risk “hotspots” were defined as multi-factor segments showing:

Very high issue prevalence, and

A minimum sample size sufficient for interpretation ($n \geq \sim 15$).

Identified Risk Hotspots

Segment	Mental health issue rate	Sample size (n)
High stress + Hybrid working	100%	17
Medium stress + Hybrid working	100%	23
High stress + On-site working + Age 36–45	96.8%	31

Interpretation

Stress level is the dominant risk driver. Once stress is high, mental health issue prevalence approaches saturation regardless of work arrangement.

Hybrid working is not inherently protective. Under medium-to-high stress, hybrid workers show issue rates equal to or higher than on-site employees.

Employees aged 36–45 under high stress represent a particularly vulnerable group, likely reflecting combined work, career, and personal pressures.

Implications for Action

Mental health interventions should prioritise stress reduction, rather than relying solely on flexible or hybrid work policies.

Hybrid workers experiencing moderate stress should be actively monitored and supported, as risk remains high before stress becomes severe.

Targeted support for mid-career employees (36–45) may yield disproportionate benefits.

Key insights for leaders (what this tells us)

High stress is the strongest driver of mental health issues. Once employees report high stress, mental health issues become very common, regardless of age or whether they work on-site or hybrid. Flexible working alone does not offset sustained pressure.

Hybrid working is not automatically protective. Employees working hybrid arrangements still show very high issue rates when stress is medium or high. This suggests that flexibility without workload control or good management support can leave problems hidden rather than solved.

Mid-career employees (ages 36–45) under high stress are a priority group. This group shows some of the highest issue rates and often carries heavy responsibility at work and at home. Intervening here is likely to reduce risk quickly and prevent longer-term absence.

Problems appear early, not only at crisis point. Even employees reporting *medium* stress show very high rates of mental health issues. This indicates an opportunity for earlier, preventative action rather than waiting until stress becomes severe.

Support systems may exist, but they are not consistently reducing risk. The data suggests that current arrangements are not reliably lowering mental health risk for

those under pressure, pointing to gaps in line-manager capability, workload clarity, or awareness of available support.

What surprised us:
The assumption that hybrid working reduces mental health risk does not hold when stress is present. Stress management and supportive leadership matter more than location.

Who should we prioritise first — and why?

Employees reporting high stress (all work modes)

Highest concentration of mental health issues

Fastest route to reducing risk, absence, and disengagement

Hybrid workers with medium-to-high stress

Issues may be less visible

Risk of problems escalating without early intervention

Mid-career employees (36–45) in high-pressure roles

Disproportionate risk

Likely to be critical to organisational performance and continuity

Recommended practical interventions

1. Strengthen line-manager capability to reduce stress at source

What this means:
Equip line managers to spot early signs of stress, have confident wellbeing conversations, and adjust workload and priorities where possible. This aligns with UK best practice on creating a *supportive working environment* rather than relying solely on individual resilience.

What to do:

Mandatory short training for people managers on stress risk, reasonable adjustments, and signposting support

Clear expectation that workload and priorities are reviewed regularly, not only during performance issues

How we'd measure impact:

Reduction in the proportion of employees reporting “high stress” (survey trend)

Improvement in average work-life balance score in follow-up surveys

2. Make support visible, normalised, and easy to access

What this means:
Ensure employees clearly understand what help is available (EAP, occupational health, mental health first aiders, flexible adjustments) and feel safe using it without stigma.

What to do:

Simple, repeated communications explaining *what support exists and when to use it*

Managers encouraged to proactively remind teams about support during busy periods

How we'd measure impact:

Improved awareness of available mental health support (survey item)

Earlier engagement with support services (e.g., increased EAP use for advice, not crisis)

Limitations and further data needed

This is self-reported survey data.
It shows strong associations, but correlation does not prove causation.

We need team, department, and job level data
This would allow more targeted interventions where pressure is structurally highest.

We need absence, turnover, and performance data
Linking wellbeing to outcomes would strengthen the business case and help track impact.

We do not yet measure workload intensity directly
Adding clearer workload and role clarity metrics would help identify root causes of stress.

Why this matters (UK context)

Workplace stress is one of the leading causes of sickness absence in the UK, with mental health-related absence continuing to rise across sectors. Employers are increasingly expected — by regulators, employees, and the public — to provide psychologically safe, supportive environments, not just flexible working options. Addressing stress early is both a people responsibility and a business necessity.